

RehabFlow — Concussion Rehabilitation Pathway

PHASE 1

Acute

Day 0–2

GOALS

Confirm diagnosis. Exclude red flags.
Prevent second impact. Establish education
+ baseline.

ASSESS

Red flag screen (→ ED if positive). SCAT6
< 72 hrs. Cervical spine screen.
Mechanism + modifying factor history.

INTERVENE

Relative rest (not cocoon). Patient / family
education. Paracetamol first-line. Sleep
priority. Day 3–5 review scheduled.

PROGRESS WHEN →

No red flag development. Family
understands plan. Cervical cleared for
movement.

PHASE 2

Early Active

Day 2–14

GOALS

Introduce sub-symptom threshold activity.
Monitor trajectory. Identify early warning
signs.

ASSESS

SCAT6 / SCOAT6 at review. Cervical re-
assess. VOMS (> 72 hrs). Symptom
trajectory vs. expected.

INTERVENE

Graded aerobic at sub-threshold.
Symptom-guided ADL progression. Sleep
hygiene. Address cervical early — biggest
win first 2 wks.

PROGRESS WHEN →

Symptoms trending down. SCOAT6
falling. Tolerating light aerobic.
Returning to work/school tasks.

PHASE 3

Graded Return

Week 2–4

GOALS

Progress activity demands. Restore work /
school / sport function. Identify patients
off the recovery curve.

ASSESS

SCOAT6 comparison vs. previous. Full
exam: cervical, VOMS, balance. Mood /
sleep screening. Functional status.

INTERVENE

Staged RTP / RTL / RTW progression.
Continue phenotype-specific rehab. BCTT-
guided aerobic if exertion intolerance.
Reinforce education.

PROGRESS WHEN →

Approaching full return (stage 4–6). OR
plateau → Phase 4 PPCS pathway.

PHASE 4

PPCS Pathway

Week 4+

GOALS

Comprehensive re-assessment. Phenotype
identification. Targeted multidisciplinary
rehab.

ASSESS

Full SCOAT6 with trajectory. Cervico-
neurological battery. VOMS + BPPV.
Autonomic (orthostatic, BCTT). PHQ-9,
GAD-7, ISI. Consider neuropsych /
polysomno / endocrine.

INTERVENE

Phenotype-directed: vestibular rehab,
vision therapy, cervical manual therapy,
BCTT programme, cognitive rehab, CBT,
sleep intervention. MDT as indicated.

PROGRESS WHEN →

Trajectory improving. Functional gains
across domains. Patient approaches full
return-to-activity.

PHASE 5

Recovery + Discharge

Full recovery

GOALS

Confirm full recovery. Clearance for all
activities. Prevent recurrence.

ASSESS

SCOAT6 within normal limits. Full exam
within normal limits. Successful full
progression through RTP/RTL/RTW ladder.
Patient-reported function restored.

INTERVENE

Final clearance document. Discharge
planning. Prevention education. Open-
door re-review if future injury or concern.

PROGRESS WHEN →

Discharged. Door remains open. Patient
educated on recurrence risk and
protective strategies.

⚠ COMMON FAILURE POINTS

- **Not assessing the cervical spine** — the most frequent single error. Cervicogenic contribution is often dominant driver.
- **RTS before RTL/RTW** — predicts prolonged recovery in adolescents.
- **Prolonged rest beyond 48 hrs** — de-conditions autonomic + vestibular systems.
- **Under-treatment of mood, sleep, pain** — drives chronicity.
- **Generic “rest and wait”** without phenotype ID — misses targeted interventions that work.
- **Premature clearance under pressure** — risks second-impact syndrome and litigation exposure.